🏥 Retirement Health Insurance Guide

Your complete roadmap to health coverage from early retirement through Medicare — updated for 2026

The Big Picture

Health insurance is one of the **biggest retirement expenses** most people underestimate. If you retire before 65, you need to bridge the gap until Medicare kicks in. Even after 65, Medicare doesn't cover everything — you'll still need a strategy for the gaps.

This guide covers three phases:

1. **Pre-65:** Bridging the gap from retirement to Medicare
2. **At 65:** Medicare enrollment and choosing a plan
3. **Ongoing:** Supplemental coverage and cost management

Phase 1: Before Medicare (Pre-65)

If you retire before 65, you lose employer coverage and need to find your own. Here are your options, ranked by value:

🏆 Option 1: ACA Marketplace Plans (Best for Most Early Retirees)

HEALTHCARE.GOV OR YOUR STATE EXCHANGE

The Affordable Care Act marketplace is usually the **best option** for early retirees. Key benefits:

- **Guaranteed coverage** — can't be denied for pre-existing conditions
- **Income-based subsidies** — premium tax credits can dramatically reduce costs
- **Essential benefits** — prescription drugs, preventive care, mental health all covered
- **Cost-sharing reductions** — lower deductibles and copays for qualifying incomes

What It Costs (2026)

Plan Tier	Coverage Level	Best For
Bronze	60% coverage	Healthy, want lowest premium
Silver	70% coverage	Best value with subsidies (get cost-sharing reductions)
Gold	80% coverage	Frequent doctor visits, prescriptions
Platinum	90% coverage	Maximum coverage, highest premium

⚠️ 2026 Subsidy Warning

The enhanced premium subsidies from the Inflation Reduction Act **expired after 2025**. In 2026:

- The 400% Federal Poverty Level income cap is reinstated
- A 60-year-old couple earning $85,000 could see premiums jump by **$20,000+ per year**
- If your retirement income is low enough, you may still qualify for significant subsidies
- **Strategy:** Control your MAGI (modified adjusted gross income) by managing Roth conversions, capital gains, and retirement account withdrawals

Best ACA Carriers

- **Blue Cross Blue Shield** — Available in all 50 states, broadest networks
- **UnitedHealthcare** — Large provider networks, good telehealth
- **Anthem** — Flexible plans, strong preventive coverage
- **Ambetter (Centene)** — Most affordable marketplace option in many states
- **Oscar Health** — Best digital/app experience

> ***Pro tip:*** *Always choose a Silver plan if you qualify for cost-sharing reductions (CSR). The savings only apply to Silver tier plans and can cut your deductible by thousands.*

Option 2: Spouse's Employer Plan

If your spouse is still working, getting on their employer plan is often the **most cost-effective option**. Employer subsidies typically cover 70-80% of the premium.

- Usually broader networks than ACA plans

- Lower out-of-pocket costs
- Losing this coverage qualifies you for a Special Enrollment Period on the marketplace

Option 3: COBRA (Short-Term Bridge Only)

UP TO 18 MONTHS OF CONTINUED EMPLOYER COVERAGE

COBRA lets you keep your exact same plan after leaving your job, but **you pay the full cost** (employer portion + your portion + 2% admin fee).

- Same doctors, same network, same plan
- No coverage gap
- **Expensive:** Typically $600-$800/month for individuals, $1,500-$2,200/month for families
- Only lasts 18 months (36 months in some cases)
- **Best use:** Bridge for a few months while setting up ACA coverage, or if you're close to 65

> ***Important:*** *Leaving COBRA qualifies you for a Special Enrollment Period on the ACA marketplace. So you can start with COBRA, then switch to ACA before it runs out.*

Option 4: Health Sharing Ministries

Not technically insurance, but an alternative some retirees use:

- Lower monthly costs (typically $200-$500/month)
- Members share each other's medical expenses
- **Risks:** Not regulated like insurance, no guaranteed coverage, can deny claims

- Pre-existing conditions often excluded for 1-3 years
- **Not recommended** as your primary strategy, but can work for very healthy people on a tight budget

Option 5: Part-Time Job with Benefits

Some employers offer health benefits to part-time workers:

- **Costco** — Benefits at 24+ hours/week
- **Starbucks** — Benefits at 20+ hours/week
- **UPS** — Benefits for part-timers
- **REI, Whole Foods, Lowe's** — Various part-time benefit options

Bonus: Earning up to $23,400/year (2025 limit) won't reduce your Social Security benefits if you're collecting early.

Phase 2: Medicare at 65

Medicare Basics

Part	What It Covers	2026 Cost
Part A	Hospital stays, skilled nursing, hospice	$0/month (if you paid Medicare taxes 10+ years)
Part B	Doctor visits, outpatient care, preventive services	$202.90/month standard premium

Part C	Medicare Advantage (private plan replacing A+B)	Varies by plan ($0-$100+/month on top of Part B)
Part D	Prescription drug coverage	Varies (~$30-$50/month average)

2026 Medicare Costs at a Glance

- **Part A deductible:** $1,736 per benefit period
- **Part B deductible:** $283/year
- **Part B coinsurance:** You pay 20% of approved services (no cap!)
- **Part B premium:** $202.90/month (higher if income > $106,000 individual / $212,000 couple)

> ***Critical:*** *Original Medicare (Parts A+B) has* ***NO out-of-pocket maximum****. A serious illness could cost you tens of thousands. That's why you need supplemental coverage.*

⏰ Enrollment Deadlines — Don't Miss These!

- **Initial Enrollment Period (IEP):** 7-month window around your 65th birthday (3 months before, your birthday month, 3 months after)
- **Late enrollment penalty:** 10% higher Part B premiums for each 12-month period you could've had coverage but didn't — *and this penalty lasts for life*
- **Exception:** If you have employer coverage at 65, you can delay enrollment without penalty

- **Medigap Open Enrollment:** 6 months starting when you're 65+ AND enrolled in Part B — **best time to get Medigap** (guaranteed issue, no health screening)

Phase 3: Choosing Your Medicare Strategy

You have two main paths once you're on Medicare:

Path A: Original Medicare + Medigap (Recommended)

BEST FOR: FLEXIBILITY, TRAVEL, PREDICTABLE COSTS

Keep Original Medicare (Parts A+B) and add a Medigap (Medicare Supplement) policy to cover the gaps.

Why This Is Usually Better

- **See any doctor** who accepts Medicare — anywhere in the country
- **No referrals needed** for specialists
- **Predictable costs** — Medigap covers most or all out-of-pocket expenses
- **Travel-friendly** — works nationwide, some plans cover foreign emergencies
- Only 17% of Medigap enrollees spend >10% of income on healthcare vs. 23% of Medicare Advantage enrollees

Best Medigap Plans for 2026

Plan	Monthly Premium (avg)	What It Covers	You Pay

Plan G ⭐	$150-$250	Almost everything	Only the Part B deductible ($283/year)
Plan N	$100-$180	Most gaps	Part B deductible + $20 doctor copay + $50 ER copay + excess charges
High-Deductible G	$40-$80	Same as G after $2,950 deductible	First $2,950/year out of pocket

> ***Plan G is the gold standard.*** *It's the most comprehensive plan available to new enrollees (Plan F was closed to new enrollees after 2020). You only pay the $283 Part B deductible per year — everything else is covered. For about $200/month, you get near-complete peace of mind.*

Best Medigap Insurance Companies

- **UnitedHealthcare/AARP** — Largest Medigap provider, widely available
- **Blue Cross Blue Shield** — Strong networks, state-specific options
- **Cigna** — Competitive rates, good customer service
- **Humana** — Well-known Medicare brand
- **Aetna** — Good rates in many markets

You still need Part D (drug coverage) separately with this path. Shop plans annually at Medicare.gov Plan Compare.

Path B: Medicare Advantage (Part C)

BEST FOR: LOW PREMIUMS, BUNDLED COVERAGE, HEALTHY PEOPLE ON A BUDGET

A private plan that replaces Original Medicare. Often includes drug coverage, dental, vision, and hearing.

Pros

- **Lower monthly premiums** — many plans are $0/month (beyond Part B premium)
- **Bundled benefits** — drugs, dental, vision often included
- **Out-of-pocket maximum** — caps your annual spending (unlike Original Medicare)
- Some plans offer gym memberships, meal delivery, transportation

Cons

- **Restricted networks** — must use plan's doctors (HMO) or pay more for out-of-network (PPO)
- **Prior authorizations** — plan can deny or delay care
- **Geographic limits** — usually tied to your local area
- **Higher costs when sick** — copays and coinsurance add up fast with serious illness
- **Hard to switch back** — may not pass health screening to get Medigap if you leave MA after the initial period

> ***The catch:*** *Medicare Advantage looks great when you're healthy. But if you get seriously ill, the copays, prior authorizations, and network restrictions can become a real problem. Many people who switch to MA regret it when they need expensive care.*

The HSA Strategy: Your Secret Weapon

💰 Health Savings Account (HSA)

If you're still working and have a High-Deductible Health Plan (HDHP), an HSA is the **most powerful tax-advantaged account** available — even better than a 401(k) for healthcare costs.

Triple Tax Advantage

1. **Tax-deductible contributions** — reduces your taxable income
2. **Tax-free growth** — invest it and let it compound
3. **Tax-free withdrawals** — for qualified medical expenses, forever

2026 Contribution Limits

- **Individual:** $4,300/year
- **Family:** $8,550/year
- **Catch-up (55+):** Extra $1,000/year

HSA in Retirement

- Can pay Medicare Part B, Part D, and Medicare Advantage premiums **tax-free**
- Can pay for dental, vision, hearing aids, long-term care premiums
- After 65, can withdraw for *any* purpose (taxed like a traditional IRA, but no penalty)
- **No required minimum distributions** — unlike 401(k)s and IRAs
- Fidelity estimates a 65-year-old couple will need **$165,000** for healthcare in retirement

> ***Best strategy:*** *If you can afford it, don't spend your HSA while working. Pay medical expenses out-of-pocket, let the*

> *HSA grow invested, and use it as a tax-free healthcare fund in retirement. Save your receipts — you can reimburse yourself years later.*

Retirement Health Insurance Checklist

✅ Before You Retire

- ☐ Estimate your healthcare costs for the gap years (age now to 65)
- ☐ Check if spouse's employer offers coverage
- ☐ Research ACA marketplace plans at Healthcare.gov
- ☐ Calculate your expected MAGI to estimate ACA subsidies
- ☐ Max out your HSA if you have one
- ☐ Understand COBRA costs from your employer (get actual numbers)
- ☐ Budget $500-$1,500/month per person for pre-65 health insurance

✅ At Age 65

- ☐ Enroll in Medicare Part A and Part B during your Initial Enrollment Period
- ☐ Choose: Medigap + Part D **or** Medicare Advantage
- ☐ If Medigap: enroll during your 6-month Open Enrollment (guaranteed issue!)
- ☐ If Medicare Advantage: compare plans at Medicare.gov

- ☐ Review and switch Part D plans annually during Open Enrollment (Oct 15 - Dec 7)

✅ Ongoing

- ☐ Review your coverage annually — plans and costs change every year
- ☐ Keep income manageable to avoid IRMAA surcharges (higher Part B premiums)
- ☐ Use HSA funds strategically for qualified medical expenses
- ☐ Consider long-term care insurance before health issues arise

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The Bottom Line

Recommended Strategy for Most Retirees

1. **Before 65:** ACA Marketplace Silver plan (manage income to maximize subsidies)
2. **At 65:** Original Medicare (A+B) + Medigap Plan G + standalone Part D
3. **Always:** Max out HSA while you can, let it grow, use it tax-free for healthcare

This combination gives you the most flexibility, the most predictable costs, and the best protection against catastrophic healthcare expenses. It's not the cheapest month-to-month, but it's the smartest long-term play.

Last updated: March 2026 | Sources: CMS.gov, Medicare.gov, Healthcare.gov, Fidelity, KFF, AARP, NerdWallet

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